



General Assembly

Seventy-eighth session

16th plenary meeting
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Official Records

President: Mr. Francis (Trinidad and Tobago)

The meeting was called to order at 10.05 a.m.

Agenda item 138 (continued)

Scale of assessments for the apportionment of the expenses of the United Nations

Report of the Fifth Committee (A/78/383)

The President: The positions of delegations regarding the recommendation of the Fifth Committee have been made clear in the Committee and are reflected in the relevant official records. Therefore, if there is no proposal under rule 66 of the rules of procedure, may I take it that the General Assembly decides not to discuss the report of the Fifth Committee that is before the Assembly today?

It was so decided.

The President: Statements will therefore be limited to explanations of position.

May I remind members that, in accordance with decision 34/401, a delegation should, as far as possible, explain its vote only once, that is, either in the Committee or in plenary meeting, unless that delegation's vote in plenary meeting is different from its vote in the Committee, and that explanations of vote are limited to 10 minutes and should be made by delegations from their seats.

Before we begin to take action on the recommendation contained in the report of the Fifth Committee, I should like to advise representatives that we are going to proceed to take a decision in the same

manner as was done in the Fifth Committee, unless notified otherwise in advance.

The Assembly has before it a draft resolution recommended by the Fifth Committee in its report.

I now give the floor to the representative of the Russian Federation.

Mr. Alyakin (Russian Federation) (*spoke in Russian*): The Russian Federation does not object to restoring the vote to the Comoros, Somalia and Sao Tome and Principe.

At the same time, Russia would like to disassociate itself from paragraphs 108, 113 and 122 of the report of the Committee on Contributions (A/78/11), in which the war in Ukraine is mentioned as a factor that affected the ability to pay of the countries that are currently in arrears. The economic problems that led to the arrears are systemic in nature and appeared long before the special military operation in Ukraine.

The President: The Assembly will now take a decision on the draft resolution entitled "Scale of assessments for the apportionment of the expenses of the United Nations: requests under Article 19 of the Charter", which was recommended by the Fifth Committee in paragraph 7 of its report.

The Fifth Committee adopted the draft resolution without a vote. May I take it that the Assembly wishes to do the same?

The draft resolution was adopted (resolution 78/2).

This record contains the text of speeches delivered in English and of the translation of speeches delivered in other languages. Corrections should be submitted to the original languages only. They should be incorporated in a copy of the record and sent under the signature of a member of the delegation concerned to the Chief of the Verbatim Reporting Service, room AB-0928 (verbatimrecords@un.org). Corrected records will be reissued electronically on the Official Document System of the United Nations (<http://documents.un.org>).



The President: The Assembly has thus concluded this stage of its consideration of agenda item 138.

Agenda items 13, 122 and 123 (*continued*)

Integrated and coordinated implementation of and follow-up to the outcomes of the major United Nations conferences and summits in the economic, social and related fields

Note by the Secretary-General (A/78/258)

Strengthening of the United Nations system

United Nations reform: measures and proposals

The President: I have been informed that no Member State has requested to take the floor under this item.

The Assembly has thus concluded this stage of its consideration of agenda items 13, 122 and 123.

Agenda item 117 (*continued*)

Follow-up to the outcome of the Millennium Summit

Specific meeting focused on development (resolution 60/265)

The President: I have been informed that no Member State has requested to take the floor under this item. The General Assembly has thus concluded this stage of its consideration of agenda item 117.

Agenda item 125

Global health and foreign policy

Draft resolutions (A/78/L.2, A/78/L.3 and A/78/L.4)

The President: Before we proceed to take action on the draft resolutions, I should like to remind members that the Assembly will hold a debate on this item on 29 November.

As indicated in my letter dated 26 September 2023, in accordance with paragraph 4 of resolution 75/315 and paragraph 5 of resolutions 77/274 and 77/275, I have submitted the political declarations approved by the high-level meetings and annexed to the present draft resolutions, for adoption by the General Assembly at this morning's meeting.

We shall now proceed to consider draft resolutions A/78/L.2, A/78/L.3 and A/78/L.4. Delegations wishing to make a statement in explanation of position before adoption on any or all of the draft resolutions are invited to do so now in one intervention.

Before giving the floor for explanations of position before adoption, may I remind delegations that explanations of position are limited to 10 minutes and should be made by delegations from their seats.

Mr. Pérez Ayestarán (Bolivarian Republic of Venezuela) (*spoke in Spanish*): The Bolivarian Republic of Venezuela has the honour to take the floor on behalf of the Group of Friends in Defence of the Charter of the United Nations, the members of which associate themselves with the statement to be made shortly by the representative of Cuba on behalf of the Group of 77 and China.

Today once again, we will fail. Once again, we will knowingly and willingly flout our pledge to leave no one behind. We will adopt not one, nor two but three documents that deliberately disregard the negative impact of unilateral coercive measures on more than a third of humankind. As we said in this very General Assembly Hall about a week ago (see A/78/PV.13), that is one of the reasons that our peoples feel frustrated with the United Nations and fail to understand why, despite its convening power and authority, our decisions do not have an impact on, or genuine influence over, their daily lives.

Under those conditions and with such a disastrous approach, seeking to carry on as normal with business as usual, how can we hope for a different and truly meaningful outcome? If we continue along the same path, allowing the same supremacist mentality to prevail in these venues, we ought to clearly say that we are doomed to keep failing.

The issue of unilateral coercive measures can no longer be ignored or brushed aside, especially amid the current situation, in which we see an increasing trend of resorting to the ongoing, methodical, relentless, unprecedented and ever-increasing adoption, implementation and even expansion of those so-called sanctions. That is having an impact on not only targeted nations, but also on the global economy, as well as supply chains and food and energy security, among other areas. As a result, unilateral coercive measures deliberately exacerbate the global multifaceted crisis facing humankind today.

We wonder whether the coronavirus disease (COVID-19) pandemic, which just officially ended less than five months ago, has taught us anything. Have we not learned the hard way, with more than 7 million lives lost, that we are as strong as the weakest link? Have we not understood that we live in an interconnected world, in which the realities and challenges faced by any given nation have repercussions on the other side of the world?

It is therefore rather alarming that the three documents that we are to adopt this morning omit such a critical issue as the question of unilateral coercive measures, which, as the facts show, and as even United Nations independent experts have established, have negative impacts on the realization of the very right to health. It would seem that we have not learned anything from the tragedy that we just experienced during the most recent pandemic, in which countries subjected to those illegal, cruel and inhumane measures were not able to have timely or effective access to vaccines, medicines and medical equipment and supplies, among other basic goods and services, in order to fight not only the COVID-19 pandemic, but also other diseases, including tuberculosis.

The moral compass of the General Assembly, we must say, seems to be lost and clearly does not seem to be in tune with the hopes of millions of people around the world, including the aspirations of the very peoples whom the United Nations claims to safeguard. It is therefore all the more disappointing to note how short the memory of the General Assembly is regarding issues that threaten the very right to life of millions of people in more than 30 countries around the world.

Since the very beginning of this process back in June, our delegations stressed, both individually and as a Group, the critical importance that they attached to including the issue of unilateral coercive measures in the draft political declarations to be formally adopted this morning. We say that it is of critical importance because many of the States members of our Group of Friends are subjected to the illegal application of such measures and know first-hand the negative and indiscriminate impact that they have on their peoples.

As has always been the case, we resolved to participate in an active and constructive manner throughout the negotiations, while being committed to reaching compromises that would ultimately allow us to achieve a consensus outcome. We engaged at all

times in good faith and, as usual, demonstrated a great deal of flexibility.

Across the table, however, we never found the same spirit and eagerness to reach compromises — quite the opposite. From the beginning, there was an attempt to impose a kind of veto on any sort of discussion on including the issue of unilateral coercive measures in the draft texts under consideration today. We must emphatically stress that that is not the way in which to conduct intergovernmental negotiations within the United Nations. We regret, on the one hand, that the co-facilitators failed to discourage such an approach throughout the negotiations and, on the other hand, that the Office of the former President of the General Assembly even attempted to impose a non-existent consensus in relation to those documents.

Throughout the almost four months of extensive negotiations, we proposed various options, alternatives and innovative formulations, even apart from the consensus language itself previously agreed in the 2030 Agenda for Sustainable Development on the matter, which should have been the easiest to incorporate in the draft text. Yet, with the exception of the process regarding universal health coverage, there was not a single attempt to include the issue of unilateral coercive measures in the draft text.

Despite that, owing to our flexibility and desire to ensure the success of the important high-level meetings held last month, we then put forward the possibility of working on a political package that could serve as a basis for reaching compromises that would ultimately lead us to adopting all those documents by consensus. However, we regret that that option was not considered in a meaningful manner either, which resulted in the concerns of a large number of delegations, including those of our Group of Friends, being ignored once again.

Finally, we conclude with the following points.

First, we regret that we were unable to achieve consensus on any of the political declarations on which we will shortly take action despite our many efforts to that end. The draft texts to be adopted, needless to say, remain incomplete, as they lack a crucial element for the realization of the right to health, namely, the urgent call for the elimination of unilateral coercive measures.

Secondly, we would like the shortcomings in those three processes, both on substance and on procedure, to be urgently redressed, including to ensure greater

transparency and prevent undue exclusions, particularly of key stakeholders, such as those that may have broken the silence on the draft documents under negotiation. We underscore the fact that imposing political agendas or consensus cannot be the norm, neither can it become the new trend or, much less so, be assumed as an accepted practice by this organ.

Thirdly, we reiterate our steadfast support and unwavering solidarity with nations and peoples subjected to unilateral coercive measures, while reiterating our strong call for their complete, immediate and unconditional lifting.

Fourthly, in strict adherence to the Charter of the United Nations and the rules of procedure of this organ, we reserve the right to carry out the actions that we deem appropriate in order to prevent that such a situation ever repeats in the General Assembly.

Mr. González Behmaras (Cuba): I have honour to deliver this statement on behalf of the Group of 77 and China in explanation of position on the adoption of draft resolution A/78/L.4.

Throughout the negotiation process of that draft text, the Group engaged in a constructive spirit and demonstrated great flexibility and a willingness to compromise. Regrettably, despite having been clear in its legitimate concerns and priorities, the Group was put in the unfortunate situation of having to break the silence twice. In that regard, the Group recalls its letter dated 1 September 2023, addressed to the President of the General Assembly.

The Group notes that most of the issues on which it broke the second silence procedure remain unsolved. Particularly on the most substantive of the Group's concerns, there was not even an attempt to find a compromise solution, and the elements objected to were pushed forward when the lack of overall consensus was crystal clear.

The Group of 77 and China comprises more than a two-thirds majority of the United Nations — 69 per cent of it, to be precise. The aspirations, priorities and legitimate concerns of developing countries should not be blatantly neglected, which is what happened when the same draft text on which our Group broke the silence was put forward as the outcome of a negotiation process that unfortunately had its shortcomings. That take-it-or-leave-it approach does not reflect the spirit of multilateralism and cooperation that should prevail

in the United Nations if it is that we truly want to leave no one behind.

It is also unacceptable and discriminatory that the co-facilitators decided to weaken agreed language of the previous political declaration on tuberculosis (TB) in an effort to accommodate one single delegation when, at the same time, they took the deliberate decision to ignore the concerns and proposals of the largest Group of the United Nations, most of them based on agreed language. That way of proceeding is not acceptable and should not create any precedent or practice in future processes.

In that regard, the Group would like to put on record its dissatisfaction with the shortcomings of the negotiation process and outcome of the 2023 political declaration on the fight against tuberculosis. That process was not conducted in a balanced and fair manner, and the delegations of developing countries were not taken into consideration on the same footing as others.

On substance, the Group would like to mention the following shortcomings of the political declaration to be adopted. First, there is no reference to unilateral coercive measures and the need to refrain from applying them against developing countries, despite it being agreed language of the 2030 Agenda for Sustainable Development. Secondly, there is no reference to the need for developing countries to have unhindered access to necessary resources and measures to address tuberculosis. Thirdly, the language on sufficient financial support for developing countries is rather weak and heavily caveated, and there is no debt dimension to the draft text. Fourthly, the language on the Trade-Related Aspects of Intellectual Property Rights is weaker than the one contained in the previous political declaration. Fifthly, the language on technology transfer is insufficient and excessively caveated.

The Group came to this process in good faith, believing that our partners were interested in an outcome document that reflected the interests and aspirations of all. Their actions during the negotiation process, even trying to delete references to the special situation of developing countries, the disproportionate impact of TB that many in our Group face, and their uneven capacity to respond to that disease, have demonstrated that, for some, leaving no one behind is just a slogan. Regardless, our Group reiterates its commitment to the fight against TB as part of the overall efforts towards implementing the 2030 Agenda.

(spoke in Spanish)

In addition to what has already been said on behalf of the Group of 77 and China with regard to document A/78/L.4, I would like to add the following in our national capacity, while associating ourselves with the statement made by the representative of Venezuela.

My country is fully committed to the efforts of the Organization to realize the right to health for all human beings. Cuba's contribution in that regard is recognized in developing countries and in forums dealing with the topic. In that spirit, my delegation was involved in its national capacity and in a very constructive and flexible manner in the three negotiating processes, the outcomes of which (draft resolutions A/78/L.2, A/78/L.3 and A/78/L.4) are to be adopted today.

My delegation wishes to register its dissatisfaction with the outcomes of the three political declarations and the way in which those processes developed towards their final outcomes. What happened around those three processes in recent weeks is of concern. Those draft texts should have been used as an opportunity to unite the efforts of the international community towards better outcomes regarding access to health for all. However, they exposed the inequities of the current international order.

The lack of political commitment on the part of some developed countries to address core issues for the South, such as issues of access to resources, knowledge and technologies or the imposition of unilateral coercive measures, which violate the Charter of the United Nations and international law, with a direct impact on the health of populations and hindering socioeconomic development, has been evident.

It is unacceptable that none of those draft texts include even a minimal reference to unilateral coercive measures, an issue of an existential nature for many developing countries, despite the fact that the language proposed time and again was even weaker than the consensus of the 2030 Agenda.

How are we to believe that it is true that no one should be left behind if the basic demand of many countries, representing the voices of millions affected by those measures in their daily lives, is completely ignored? How can we trust that there is a genuine will to move forward together with regard to health if great efforts are made to accommodate the interests of rich countries, while the legitimate concerns of developing

countries, or even the Group of 77 and China as a whole, are underestimated and silenced?

The lack of transparency, balance, impartiality and fairness in the way in which some of those processes have developed has also been evident, as has the lack of activism from the Office of the President of the General Assembly. There has been everything — from facilitators who took advantage of their position to favour their own agendas and refused to include developing countries' demands, to attempts to accommodate specific countries, weakening the agreed language of interest to the South.

The facilitators of the universal health coverage process deserve special mention and recognition. They made huge efforts to include and preserve to the end language of great importance to the developing world.

We understand that the facilitators, appointed by the Office of the President of the General Assembly, serve under the authority of the General Assembly and are accountable to it. In any case, the Office is responsible for the draft texts submitted for consideration by the Assembly after such facilitation processes, and it has full authority to find political or compromise solutions in the absence of consensus.

My delegation would like to recall that, both in its national capacity and on behalf of the Group of 77 and China, it conveyed to the Office a willingness to continue negotiations towards a political solution to the situation arising around all those draft texts. We held meetings with the previous and the current Office and made clear our concerns and priorities. We proposed the option of exploring a compromise package. However, it became clear that, in today's United Nations, some countries are more listened to and accommodated than others regarding their priorities, which is unacceptable.

It is also unacceptable to impose consensus that has not been reached, to limit negotiations to issues of interest to developed countries and that such countries have a certain veto power over issues that are not addressed in the negotiations. There should be no selective and discriminatory treatment in an Organization founded on sovereign equality.

Moreover, my delegation recalls that, on 17 September, 11 Member States sent a letter to the President of the General Assembly, outlining concerns regarding those processes and requesting that it be circulated as an official document under agenda items

19 and 127. It is of concern that, 18 days later, that letter has not been circulated.

In conclusion, my delegation would like to indicate that it does not endorse the way of proceeding with the draft texts, or does it consider it a precedent or practice for the future.

Mr. Manyanga (Zimbabwe): My delegation aligns itself with the statement just delivered by the representatives of Cuba, on behalf of the Group of 77 and China, and of Venezuela, on behalf of the Group of Friends in Defence of the Charter of the United Nations. Allow me to make a few further remarks in my national capacity.

In that regard, let me start by expressing the appreciation of my delegation to the co-facilitators of the three health-related political declarations (draft resolutions A/78/L.2, A/78/L.3 and A/78/L.4), to be adopted by the General Assembly today, for their leadership of the processes that culminated in the adoption of the political declarations of the three high-level health-related meetings. The subject matters under consideration, namely, pandemic prevention, preparedness and response, universal health care and the fight against tuberculosis (TB), are very relevant and timely.

While joining the consensus on the adoption of those important draft resolutions, which express the commitment and dedication of the international community to address the ever-increasing global health-related threats, my delegation wishes to express serious concern over the exclusion from the three political declarations of language that acknowledges and recognizes the negative impact of unilateral coercive measures on the enjoyment of the right to good health and, ipso facto, to the implementation of the Sustainable Development Goals (SDGs). The language was overwhelmingly supported by Member States from across the regions and is agreed language in the 2030 Agenda for Sustainable Development. That omission is of profound consequence, given that unilateral coercive measures affect a significant proportion of the global population, and undermines our shared commitment to leaving no one behind. The economic hardships caused by unilateral coercive measures hinder the ability of Governments to mobilize adequate resources to provide essential health-care services, depriving vulnerable populations of critical medical care, including access to vaccines, medications and medical

equipment. Unilateral coercive measures undermine, and constrain, the ability of the countries concerned to adequately prepare for, prevent and respond to pandemics, to achieve universal health coverage and, of course, to win the fight against tuberculosis.

Unilateral coercive measures have an impact on the rights and lives of a third of the world population, mainly innocent civilians in countries that are victims of the heinous, illegal and unwarranted measures. To testify to that reality, the General Assembly appointed a Special Rapporteur on the negative impact of unilateral coercive measures on the enjoyment of human rights. In that regard, contrary to claims by those that resort to unilateral coercive measures, the United Nations recognizes that the measures indeed impact the human rights of victim populations. The principle of leaving no one behind will remain a mere slogan if we relegate the issue of the negative, multidimensional impacts of unilateral coercive measures to the periphery of what we do.

The political declarations are an opportunity to address all major issues militating against the global efforts to achieve universal health coverage, fight against tuberculosis and prepare for, prevent and respond to future pandemics. The related high-level meetings were an opportunity to review progress on the commitments made in the 2019 political declaration on universal health coverage and in the fight against TB, with a view to addressing gaps, taking corrective action to restore progress in that regard. Unilateral coercive measures are an impediment to the attainment of the SDGs by design, as they are meant to cripple the targeted Governments.

The commitment of the United Nations to leaving no one behind is a core principle of our collective efforts to achieve the SDGs. Unilateral coercive measures have deepened inequalities and exacerbated health disparities, directly contravening that commitment. In an interconnected world, it is imperative to recognize the importance of global health security, as pandemics respect no boundaries. The exclusion of millions of sanctioned populations undermines our collective capacity to respond effectively to pandemics and other health crises. Suffice to recall that no one is safe until everyone is safe.

Health is a fundamental human right, as affirmed in the Universal Declaration of Human Rights. Therefore, the imposition of unilateral coercive measures is a serious violation of those rights. In that regard, we

reiterate our call for the unconditional removal of all illegal unilateral coercive measures, especially in the context of global health emergencies, to ensure that no nation or its citizens are left without the necessary resources to respond effectively to health crises.

In conclusion, we wish to reaffirm our commitment to the principles of solidarity, cooperation and the protection of human rights. We sincerely hope that the international community will not miss another opportunity to be inclusive. To truly leave no one behind, we must address the impact of unilateral coercive measures on global health and take decisive actions to alleviate the suffering of those affected.

Ms. Ershadi (Islamic Republic of Iran): My delegation aligns itself with the statement made by the representative of the Bolivarian Republic of Venezuela on behalf of the Group of Friends in Defence of the Charter of the United Nations and the statement delivered by the representative of Cuba on behalf of the Group of 77 (G-77) and China. We would like to present our statement in our national capacity in explanation of position on draft resolutions A/78/L.2, A/78/L.3 and A/78/L.4, and the three health-related political declarations annexed thereto, under this agenda item.

The United Nations, as per the Charter, is based on the sovereign equality of all its Member States. The views and priorities of Member States matter and should be highly respected. Nonetheless, during the negotiation of the political declarations and the onward processes we witnessed that that very fundamental principle was frequently disregarded. The priorities of a few were accommodated to the detriment of many other Member States and even large groups of developing countries. Agreed language was altered for the benefit of a few, while the proposals of developing countries were ignored. In general, one-sidedness, deliberate obscurity and ignorance unfortunately prevailed over neutrality, transparency and inclusiveness.

My delegation engaged in the negotiation of the health-related draft political declarations in good faith and in a spirit of flexibility, expecting that there would be transparent, inclusive and fair processes that would allow an outcome with which every delegation would be generally satisfied. Nevertheless, the results, in particular given the exclusion of one of our important priorities, proved otherwise. Those circumstances made many Member States, including large groups of developing countries, break the silence procedure,

in many instances twice. The statement delivered on behalf of the G-77 and China regarding the political declaration on tuberculosis is demonstrative of the extent of frustration and discontent suffered by two thirds of the United Nations as a result of that circumstance.

As a case in point, and of particular importance to my delegation, I would like to refer to the unilateral coercive measures, of which the deleterious and severe negative impacts on the health of people are axiomatic, are reflected in many relevant United Nations documents and, more importantly, are felt directly on a daily basis by innocent people who lack access to medicine, medical products and vaccines as a result of such unlawful measures. As evinced by the Special Rapporteur on the negative impacts of unilateral coercive measures on the enjoyment of human rights, unilateral coercive measures result in the violation of all categories of economic, social and cultural rights, including the rights to life, health, food, an adequate standard of living, education and the right to development.

Such detrimental impacts affect the lives, health and physical and mental well-being of the affected people, including women, children and the elderly. In addition, unilateral coercive measures undermine the ability of targeted States to meet their obligations and commitments in the area of health, while diminishing the resources and capacities in the health sector and beyond. Therefore, a progressive, concise and action-oriented text on health should indeed address this pressing challenge, which, above all other external impediments and barriers, has a dramatic and obvious negative impact on the health of people.

To just mention an instance of such impacts, I would like to refer to the situation caused by unilateral coercive measures for patients with epidermolysis bullosa, a chronic skin disease that also affects children, causing the skin to be fragile and to blister very easily. A certain medical products manufacturer stopped all exports to the Islamic Republic of Iran as a result of unilateral coercive measures in 2018. As reflected in the report of the Special Rapporteur on the negative impacts of unilateral coercive measures on the enjoyment of human rights, the halt in exports

“including bandages made only by that company which were vital for patients with epidermolysis bullosa, a horrendous chronic skin disease. This led to greater suffering and even deaths among

those children, compromising their rights to health and to life” (A/HRC/51/33, *para.* 26)

Many similar barriers to accessing, procuring and delivering life-saving medicines and medical devices to treat other rare and severe diseases exist due to the unilateral coercive measures.

Unilateral coercive measures violate human rights and, unfortunately, continue to inflict immeasurable harm on the health of people. Nothing can ever justify such horrific measures against people. Nothing can justify the death of a child who could simply not have a medical product due to unilateral coercive measures. Yet again, during the negotiations we visibly observed a few States justifying such unlawful measures, while trying to avoid any discussion that would reveal how inhumane such measures are. My delegation, along with many other delegations that favoured the inclusion of language addressing that acute challenge in the three draft political declarations, constructively engaged in the negotiations and proposed relevant language on the topic, with the idea of improving the text of the declarations by giving due regard to one of the major health challenges resulting from such unlawful measures. But even the agreed language on the unilateral coercive measures, derived from relevant documents that had been accepted by all States Members of the United Nations, was not respected.

The extent of frustration among many Member States on the way in which the three health-related processes have been handled is visible. But, unlike a certain few Western countries that are the reason for that frustration and situation, we do not intend to be part of the problem; rather, given the underlying status of the three political declarations in the health context, we have decided to join the consensus on the present draft resolutions on the understanding that such unfair, non-transparent and one-sided processes will not set any precedence in future and will not happen again. With that in mind, allow me to touch upon a few points as to the substance of the three political declarations, respectively.

We reaffirm the importance of national ownership and the primary role and responsibility of Governments at all levels to determine their own path towards achieving universal health coverage and pandemic prevention, preparedness and response, as well as in the area of tuberculosis, in accordance with their national laws, contexts and priorities. We further emphasize international cooperation, in particular in supporting

and strengthening the capacity of developing countries, including through enhanced financial development assistance, as well as financial and technical support, upon request, as well as ensuring unhindered access for developing countries to quality and affordable medicines, medical products and health technologies.

In conclusion, we would like to clarify a position regarding the present draft resolutions and the political declarations on pandemic prevention, preparedness and response, universal health coverage and tuberculosis annexed thereto, which will be adopted by the General Assembly today under this agenda item. In that respect, we would like to reiterate that the consideration and implementation of the provisions of the political declarations by the Islamic Republic of Iran are subject to, and contingent upon, its national laws, regulations, policies and priorities, as well as its religious, cultural and social specificities and values.

Mr. Baghdadi (Syrian Arab Republic) (*spoke in Arabic*): At the outset, my country's delegation aligns itself with the statements delivered by the representatives of Cuba and Venezuela on behalf of the Group of 77 and China and of the Group of Friends in Defence of the Charter of the United Nations, respectively. I would like to make the following comments in my national capacity.

My country's delegation supports the convening of the high-level meetings on health in the light of the increasing health challenges in our world. Those meetings are timely, since we need to enhance health systems and ensure their ability to deal with emerging challenges. We also need to continue to work on achieving the Sustainable Development Agenda.

My delegation engaged in the negotiation process from its inception in an effective and constructive spirit in order to reveal the negative impact of unilateral coercive measures on the health sector. We expressed our concerns, and we requested the co-facilitators to take them into account. However, some Western countries insist on politicizing the work of the Organization and refuse to engage with us owing to their destructive approach and political unilateralism. Unfortunately, the concerns of several delegations were not taken into account during the negotiation process. Despite our repeated demands, although we showed maximum flexibility and later broke the silence procedure, all that was not enough to achieve a genuinely inclusive process. Our rightful demand was ignored, and no

language pertaining to unilateral coercive measures and their negative impact on the health sector was included.

We cannot consider that such declarations are consensual. They leave a large part of populations behind because they cannot receive sustained health care due to unilateral coercive measures. In my country, Syria, the health sector was greatly affected, and a large number of our medical devices are no longer operational because we cannot import the spare parts to repair them or because the foreign companies manufacturing those devices are afraid of being sanctioned. They do not agree to repair or maintain such devices. Those measures also make us unable to provide vaccinations and medicine. That has increased the plight of the Syrian people, especially after the terrorist war that is imposed on my country and the destructive earthquake that happened on 6 February.

The unilateral coercive measures imposed on the Syrian people, the most recent of which was adding to the list of sanctions several Syrian hospitals that provide free services to the Syrian people, prevent access to the necessary preventive diagnosis, treatment devices and spare parts. The lack of morality and inhumanity also led to imposing sanctions on child-care centres. Today Western companies cannot export the necessary medical devices to treat children. The health sector in Syria is under strain owing to a decade of terrorist war, health emergencies and natural disasters.

The catastrophic consequences of unilateral coercive measures spare no sector. Today we also lack energy resources, which are negatively affecting electricity and the provision of power for hospitals and medical clinics. That makes it difficult to provide various sectors in need with basic health care and also affects the water and sanitation sectors, increasing the possibility of the outbreak of diseases and pandemics due to the lack of clean water.

It is therefore important to highlight the plight of the populations targeted by such unilateral coercive measures, especially in the health sector. To that end, my delegation requested that such language about unilateral coercive measures be added to at least one of the three political declarations. Unfortunately, the facilitators did not take the concerns of a great number of countries into account, while Western countries still ignore the catastrophic impact of unilateral coercive measures on the daily lives of the peoples of targeted countries.

Mr. Jiménez (Nicaragua) (*spoke in Spanish*): Our delegation aligns itself with the statements made in explanation of position by the representative of Cuba, on behalf of the Group of 77 and China, and by the representative of Venezuela, on behalf of the Group of Friends in Defence of the Charter of the United Nations.

The lack of good faith and will in relation to the draft texts to be adopted in the Assembly today is regrettable. Through pressure, Western aggressor countries obstructed and excluded the language of the 2030 Agenda for Sustainable Development and the paragraph on illegal and terrorist unilateral coercive measures in those texts. No less than a week ago, we witnessed a similar scenario, in which the positions of a significant group of countries were repeatedly ignored throughout the negotiation process of those declarations.

The countries of the global South continue to be hostage to the pressure and blackmail policies of imperialist and neocolonialist countries so that unilateral coercive measures are not included in issues of the utmost importance to our developing countries. We reiterate that those three declarations remain incomplete and exclude millions upon millions of people who are being affected by such illegal measures.

Let the aggressors, those that impose the illegal terror sanctions, explain how the international cooperation and solidarity that is so touted towards achieving universal health coverage are furthered under blockades, sanctions and other aggression, and how our countries can prevent, prepare for and respond to a pandemic when, during the very recent coronavirus disease, countries affected by such terrorist measures could not access vaccines, medicines, instruments and medical equipment to deal with that disease, which continues to cause so many deaths.

Health is a right that should be exercised in its entirety by all the peoples of the world, without exclusions of any kind. We reiterate that unilateral coercive measures are incompatible with, and contrary to, the Charter of the United Nations and the joint efforts of the United Nations for the implementation of the 2030 Agenda. We will therefore not cease in our call and our efforts for those illegal measures to be eliminated.

Mrs. Mozgovaya (Belarus) (*spoke in Russian*): We align ourselves with the statement made by the representative of Venezuela on behalf of the Group of Friends in Defence of the Charter of the United Nations.

We would like to make the following statement in our national capacity.

Universal health coverage is fundamental to the achievement of all the Sustainable Development Goals, while the fulfilment of all the Goals and targets of the 2030 Agenda for Sustainable Development determines the prospects for healthy lives and well-being for all. That interdependence requires the capacity-building of all countries, especially developing countries, in order to build effective national health systems and ensure that the world's population has access to quality health care.

Belarus regrets that the political declarations to be adopted today do not reflect the negative impact of sanctions policies on the functioning of the national health systems of affected States. Countries under the pressure of sanctions face shortages of medical supplies, equipment and medicines, which cannot but affect the quality of care provided to the population. Unilateral coercive measures limit their access to the medicines and medical equipment needed to treat serious diseases, including the tuberculosis infection.

In the context of a pandemic, sanctions have literally become a lethal weapon of Western countries, limiting access for affected States to medicines, vaccines, medical technology, research and development and significantly reducing their ability to provide quality health care for their populations. Today, with the coronavirus disease pandemic having receded, leaving scars in terms of serious social and economic consequences, the prospects for the long-term recovery of States are clouded by the burden of the pressure of illegal sanctions in addition to the impact of other contemporary crises.

Belarus condemns in the strongest terms the practice of unilateral coercive measures and calls for their elimination, which, among other things, would help to strengthen the global capacity to prevent and combat pandemics. We advocate strengthening the central role of the World Health Organization in the global health architecture. We expect the World Health Organization to take into account the negative impact of sanctions in its work and to focus its efforts not only on mitigating the severe humanitarian consequences of unilateral coercive measures, but also on actively countering them.

In the face of unprecedented pressure on the health system, Belarus will continue to ensure universal health

coverage and access to quality health care, medicines, vaccines and medical devices in order to strengthen global efforts to build a healthier world.

Mr. Zhang Tianhao (China) (*spoke in Chinese*): China supports the statements made by the representatives of Cuba, on behalf of the Group of 77 and China, and of Venezuela, on behalf of the Group of Friends in Defence of the Charter of the United Nations.

The coronavirus disease pandemic has once again demonstrated the importance of strengthening global public health. China welcomes the convening of the three high-level meetings — on pandemic prevention, preparedness and response, universal health coverage and the fight against tuberculosis, respectively — by the General Assembly at its seventy-eighth session, supports the Assembly's adoption of the political declarations by consensus under the relevant draft resolutions (A/78/L.2, A/78/L.3 and A/78/L.4) and is willing to join the consensus on those three political declarations.

Throughout the process of consultation, together with like-minded countries, China advocated for the inclusion of the elements of unilateral coercive measures in the political declarations. Those measures contravene international law and the purposes and principles of the United Nations Charter, severely undermine the advancement of public health and harm the people's lives and health in the sanctioned countries.

The latest report of the Special Rapporteur of the Human Rights Council on unilateral coercive measures (A/HRC/51/33) once again makes clear the serious negative impact of those measures on the right to health. However, due to the opposition of some countries, the relevant elements could not be included in the outcome documents, which China deeply regrets.

China will continue to work with like-minded countries on the platform of the United Nations to highlight the serious harm caused by unilateral coercive measures, and we will continue to strongly urge the countries concerned to abolish such measures as soon as possible.

Mr. Chumakov (Russian Federation) (*spoke in Russian*): The Russian Federation will join the consensus on the three political declarations on health (draft resolutions A/78/L.2, A/78/L.3 and A/78/L.4).

I would like to acknowledge the efforts of the coordinators of the consultations on universal health coverage, the Permanent Representatives of Guyana and

Thailand, who helped to achieve a balanced text. The outcome document contains many elements important to our common future that will truly contribute to ensuring the provision of a wide range of health care to everyone.

The world continues to move steadily towards a more equitable multipolar system, based on the principles of trust and equal opportunities for the free development of all countries and peoples. During the consultations, the majority of States clearly demonstrated their rejection of the policy of involving them in serving Western interests, including through attempts to politicize the negotiations by means of the Ukrainian issue.

Russia fully supports and associates itself with the statement made by the representative of Venezuela on behalf of the Group of Friends in Defence of the Charter of the United Nations. Our Heads of State and Government have repeatedly reaffirmed their commitment to the ideas contained in the 2030 Agenda for Sustainable Development, with a view to its full realization. That was also reflected in the discussions at the high-level meetings of the General Assembly on health. In that regard, we are disappointed that today's documents are to be adopted without paragraph 30 of the 2030 Agenda, calling for the lifting of unilateral sanctions, being included in their texts.

We are concerned about attempts to silence or simply dismiss the plight of millions of people in developing countries under sanctions. It is clear that it is not so much the crises currently being widely discussed as the illegal unilateral measures, especially extraterritorial measures aimed at depriving people of their basic needs and the right to health, that are blocking the development of States that are among the authors of the texts.

I would like to say a few words on the negotiation process. We are compelled to note the lack of neutrality and impartiality shown by the facilitators. For example, in the declaration on the fight against tuberculosis, at the request of some delegations, an entire paragraph on a comprehensive family-centred approach to the treatment of that disease was suddenly deleted. That paragraph was taken entirely from agreed language in the 2018 first political declaration on tuberculosis. It is very important and is still relevant today because it takes into account the vulnerable situation of children suffering from that infectious disease. For that reason,

it garnered significant support in the consultations room from developing States.

It was also significant that no silence procedure was successful. Ultimately, the texts were simply handed over to the President of the General Assembly for further endorsement in the Assembly. The final document of the high-level meeting on pandemic prevention, preparedness and response requires serious revision. I emphasize that only about 19 per cent of the text was agreed by Member States.

Despite the shortcomings that we have noted in the process and substance of the documents to be adopted today, Russia believes that their implementation, with the joint efforts of the entire international community, will be of considerable benefit to global public health. At the same time, to further improve the effectiveness of multilateral cooperation, among other things, the lessons learned from the processes that have been completed should be taken into account.

The President: We have heard the last speaker in explanation of position before adoption. The Assembly will now take a decision on draft resolution A/78/L.2, entitled "Political declaration of the General Assembly high-level meeting on pandemic prevention, preparedness and response".

I now give the floor to the representative of the Secretariat.

Ms. Sharma (Department for General Assembly and Conference Management): The present oral statement is made in the context of rule 153 of the rules of procedure of the General Assembly. The present statement was distributed to Member States.

Under the terms of paragraphs 77 and 78 of draft resolution A/78/L.2, the General Assembly would

"[r]equest the Secretary-General to provide, in consultation with the World Health Organization and other relevant agencies, a report including recommendations on the implementation of the present declaration towards strengthening pandemic prevention, preparedness and response during the seventy-ninth session of the General Assembly, which will serve to inform a high-level meeting to be convened in 2026;

and

"decide to convene a high-level meeting on pandemic prevention, preparedness and response

in 2026 in New York, aimed at undertaking a comprehensive review of the implementation of the present declaration, the scope and modalities of which shall be decided no later than the seventy-ninth session of the General Assembly, taking into consideration and in full coordination with the outcomes of other ongoing related processes to strengthen pandemic prevention, preparedness and response.”

The request contained in paragraph 77 of the draft resolution would constitute an addition to the documentation workload for the Department for General Assembly and Conference Management of one pre-session document with a word count of 8,500 words in all six languages in 2025, and entail additional non-recurrent resource requirements in the amount of \$24,500 in 2025.

With regard to the high-level meeting on pandemic prevention, preparedness and response in 2026 in New York, mentioned in paragraph 78 of the draft resolution, in the absence of modalities for the high-level meeting, it is not possible at the present time to estimate the potential costs implications of the requirements of the high-level meeting. When information on the format, scope and modalities of the high-level meeting becomes available, the Secretary-General will assess the budgetary implications and advise the General Assembly in accordance with rule 153 of the rules of procedures. Furthermore, in accordance with established practice, the date of the high-level meeting would be determined in consultation with the Department for General Assembly and Conference Management.

Reference is made to paragraph 11 of resolution 69/250 and subsequent resolutions, the most recent of which is resolution 77/255, in which the Assembly invited Member States to include in new legislative mandates adequate information on the modalities for the organization of conferences or meetings.

Accordingly, should the General Assembly adopt draft resolution A/78/L.2, additional resource requirements in the amount of \$24,500 under section 2, General Assembly and Economic and Social Council Affairs and Conference Management, would be included in the proposed programme budget for 2025 for the consideration of the General Assembly at its seventy-ninth session.

The President: May I take it that it is the wish of the General Assembly to adopt draft resolution A/78/L.2?

Draft resolution A/78/L.2 was adopted (resolution 78/3).

The President: The Assembly will now take a decision on draft resolution A/78/L.3, entitled “Political declaration of the high-level meeting on universal health coverage”.

I now give the floor to the representative of the Secretariat.

Ms. Sharma (Department for General Assembly and Conference Management): The present oral statement is made in the context of rule 153 of the rules of procedure of the General Assembly. The present statement was also distributed to Member States.

Under the terms of the operative paragraph 109 of draft resolution A/78/L.3, the General Assembly would

“decide to convene a high-level meeting on universal health coverage in 2027 in New York, aimed at undertaking a comprehensive review on the implementation of the present declaration to identify gaps and solutions to accelerate progress towards the achievement of universal health coverage by 2030, the scope and modalities of which shall be decided no later than the eightieth session of the General Assembly, taking into consideration the outcomes of other existing health-related processes and the revitalization of the work of the General Assembly.”

With regard to paragraph 109 of the draft resolution, in the absence of modalities for the high-level meeting in 2027 in New York, it is not possible at the present time to estimate the potential costs implications of the requirements of the high-level meeting. When information on the format, scope and modalities of the high-level meeting becomes available, the Secretary-General will assess the budgetary implications and advise the General Assembly in accordance with rule 153 of the rules of procedures. Furthermore, in accordance with established practice, the date of the high-level meeting would be determined in consultation with the Department for General Assembly and Conference Management.

In that regard, reference is made to paragraph 11 of resolution 69/250 and subsequent resolutions, the most recent of which is resolution 77/255, in which the Assembly invited Member States to include in new legislative mandates adequate information on

the modalities for the organization of conferences or meetings.

The President: May I take it that it is the wish of the General Assembly to adopt draft resolution A/78/L.3?

Draft resolution A/78/L.3 was adopted (resolution 78/4).

The President: The Assembly will now take a decision on draft resolution A/78/L.4, entitled “Political declaration of the high-level meeting on the fight against tuberculosis”.

May I take it that it is the wish of the General Assembly to adopt draft resolution A/78/L.4?

Draft resolution A/78/L.4 was adopted (resolution 78/5).

The President: Before giving the floor for explanations of position after adoption, may I remind delegations that explanations of position are limited to 10 minutes and should be made by delegations from their seats.

Ms. Jimenez de la Hoz (Spain) (spoke in Spanish): I have the honour to speak on behalf of the European Union and its member States. The candidate countries Albania, Bosnia and Herzegovina, Montenegro, North Macedonia, the Republic of Moldova, Serbia, Türkiye, Ukraine; the potential candidate country Georgia; and the European Economic Area countries Iceland, Liechtenstein and Norway, as well as Andorra, Armenia, Azerbaijan, Monaco and San Marino, align themselves with this statement.

We are pleased to join the consensus affirming the adoption of the three political declarations by our leaders at the high-level meetings held under the auspices of the General Assembly a fortnight ago (resolutions 78/3, 78/4 and 78/5).

We wish to commend the skilful co-facilitation and tireless efforts of Morocco, Israel, Guyana, Thailand, Poland and Uzbekistan, which engaged deeply with all delegations and brought us together to reaffirm our commitment to achieving Sustainable Development Goal (SDG) 3 and, more broadly, to improving the global health architecture and global health in support of sustainable development and the achievement of the goals of the 2030 Agenda for Sustainable Development.

The European Union engaged constructively and proactively in the negotiations, showing consideration

and flexibility towards other delegations’ positions and prioritizing the needs of developing countries, the human rights imperative, increasing global equity and leaving no one behind. The European Union and its member States fully participated in the high-level meetings at both the Heads of State and Government and the ministerial levels. We make a clear and unconditional commitment to effectively implementing those political declarations. They provide us with road maps for the path to 2030.

On pandemic prevention, preparedness and response, our leaders underlined the power of solidarity and collaboration and the fact that we need to urgently address the serious gaps in equity and the capacity of all to prevent, prepare for and respond to pandemics. We recall that the European Union leads by example, including through increased support for regional and local manufacturing of medical countermeasures and by providing funds to the Pandemic Fund. The European Union has chosen to take concrete steps that help to make us all safer. In addition, we are committed to another option — to also strengthen global governance with an ambitious agreement on pandemics in order to protect citizens around the world. We led the convening of negotiations and put forward ambitious proposals in the belief that such an agreement would be for the common good. For it to succeed, we must all embrace it equally as a shared responsibility. We must close the gaps in the entire cycle of prevention, preparedness and response. The declaration provides welcome political impetus to those efforts, but without prejudging the positions that members have or will have on specific issues.

Universal health coverage is one of the top priorities of the European Union Global Health Strategy. European Union leaders highlighted five points at the high-level meeting: the need to focus on primary health care and sexual and reproductive health as the basis of resilient and responsive health-care systems, the importance of essential public health functions that address the determinants of health, as well as inequities and gender equality, and the centrality of an effective health workforce in reaching the most underserved and those in the most vulnerable situations, which must be our collective priority. They stressed the power of digitalization and voluntary technology transfer on mutually agreed terms, with due respect for international intellectual property rules. They underlined that armed conflicts add to the already daunting challenges that we face today and challenge the achievement of the SDGs, including SDG

3. Our leaders also recalled that, while the European Union and its member States have committed significant funds to support our partners in strengthening their health systems, that cannot replace sound national health financing strategies, which depend on priority setting, robust governance and the mobilization of national public and private resources. More broadly, all stakeholders need to be fully engaged to build a social compact that delivers better health for all.

Although it joined the consensus, the European Union would ask to put on its dissociation on two points: first, the paragraph containing non-consensual references to the right to development and, secondly, that the technology transfer for which we call must be voluntary and on mutually agreed terms, which is long-established and agreed language that we regret was omitted from the final text, and must comply with international obligations.

Mr. Iravani (Islamic Republic of Iran), Vice-President, took the Chair.

Our leaders also voiced their full support for the eradication of tuberculosis. As a long-standing investor in tuberculosis research and development and a contributor to the Global Fund and the work of the World Health Organization, the European Union is fully committed to reaching the target set out in SDG 3.3. We believe that we must ensure access to treatment through primary health care and vital health services. We must target those in the most vulnerable and marginalized situations, who are the most affected by tuberculosis, and end the stigma. We must invest in early prevention and detection, especially for at-risk populations, promote testing and improve diagnosis and surveillance, as well as addressing the scourges of discrimination and stigmatization that hamper our best efforts. We also stress the need for new diagnostics and treatments for drug-resistant tuberculosis, as well as improving coverage and equity through digital solutions and innovative service delivery. More broadly, all stakeholders must be fully engaged.

It goes without saying that, in implementing the political declarations, the European Union calls for a strong reflection of the core values of the United Nations so as to protect, promote and fulfil the human rights of all, without discrimination of any kind, and to help to achieve gender equality and the empowerment of all women and girls.

Mr. Larsen (Australia): I take the floor on behalf of Canada, New Zealand and my own country, Australia (CANZ) regarding the three political declarations adopted today on pandemic prevention, preparedness and response (resolution 78/3), universal health coverage (resolution 78/4) and the fight against tuberculosis (resolution 78/5).

At the outset, I wish to thank the Permanent Representatives of Israel, Morocco, Guyana, Thailand, Poland and Uzbekistan and their teams for their facilitation of those declarations.

The three political declarations are focused on advancing positive developments to help to improve the health of everyone, everywhere. Their objectives are to ensure that, first, our health systems are equitable, resilient, sustainable and better positioned to prevent, prepare for and respond to pandemics and strengthen our global health architecture; secondly, that all people can access quality and equitable health services when and where they need them, without financial hardship, and consistent with human rights; and, thirdly, that an effective vaccine for tuberculosis is developed and efforts are redoubled to eradicate tuberculosis everywhere. Those objectives all align with the principles to which we all committed in the 2030 Agenda for Sustainable Development and its Sustainable Development Goals.

The coronavirus disease pandemic had devastating health, economic, political and security impacts on our societies. It also made obvious that our systems to prevent, prepare for and respond to future pandemics are inadequate. Access to essential services faltered, including for sexual and reproductive health and for the detection and treatment of cancer, HIV and other serious conditions and diseases, including tuberculosis.

Climate change is also having an impact on the delivery of health services, and increasingly frequent extreme weather events are causing disasters, death, disease, poverty and illness.

Non-communicable diseases are rising globally, and too often people are missing out on health services due to their gender, ethnicity or disability. That includes support for equitable sexual and reproductive health services. Those critical life-saving services are core to safety, good health and empowerment, particularly for women and girls in all their diversity.

On such critical issues, I note that CANZ remains deeply disappointed by the weakened language on gender, human rights and access to sexual and reproductive health services in the final versions of the political declarations.

We trust that the adoption of the political declaration on pandemic prevention, preparedness and response adds political momentum to the negotiations currently under way in Geneva for a new legally binding pandemic instrument and to amend our existing International Health Regulations.

We trust that the adoption of the political declarations on universal health coverage and on the fight to end tuberculosis spurs action to address the uneven impact that health crises have on women, people in poverty and the most marginalized, as well as to advance equality and inclusion and meet the needs of all people.

Mr. Al Rubkhi (Oman) (*spoke in Arabic*): It is my honour to deliver this statement on behalf of the delegations of the States members of the Gulf Cooperation Council, namely, the United Arab Emirates, the Kingdom of Bahrain, the Kingdom of Saudi Arabia, the State of Qatar, the State of Kuwait and my country, the Sultanate of Oman.

At the outset, our countries appreciate the efforts made to facilitate the intergovernmental negotiations and to reach the final text for the political declaration of the General Assembly high-level meeting on pandemic prevention, preparedness and response (resolution 78/3). That political declaration is the first of its kind and is a turning point in the way in which the international community deals with health pandemics. The coronavirus disease (COVID-19) pandemic has proved that its impact goes beyond health consequences and actually reaches economic, social and other aspects. Such pandemics greatly hinder the ability of States to move forward with the implementation of sustainable development plans.

The political declaration on pandemic prevention, preparedness and response has addressed a number of important and vital issues, such as the promotion of regional and international cooperation and solidarity, while encouraging multilateralism and addressing health inequality within and among States. It also builds on lessons learned and best practices learned from COVID-19. The political declaration recognizes the need to support developing countries in building

their national and domestic capacities, as well as regional capacities in that regard. It also ensures timely access to, and reasonable and fair prices for, medical material, including, inter alia, vaccines, diagnostics and treatment equipment. Moreover, the political declaration commended the important role of health-care workers in responding to pandemics, as well as their dedication and sacrifices. It encourages national, regional and international efforts to commemorate that role.

Our countries' delegations joined the consensus on resolution 78/3, on the political declaration of the high-level meeting on pandemic prevention, preparedness and response, because we believe in the importance of the issues addressed in that declaration. However, with regard to paragraph 52 of the text and the terms used, such as sexual and reproductive health-care services and reproductive rights, our countries interpret that paragraph in line with our cultural and societal frameworks and in accordance with the national laws and regulations of our countries.

Mrs. Kuzee (Namibia): My delegation wishes to make this explanation of position after the adoption of the political declarations on pandemic preparedness, prevention and response (resolution 78/3), universal health coverage (resolution 78/4) and the fight against tuberculosis (resolution 78/5), respectively.

We thank the former President of the General Assembly, as well as the respective co-facilitators and their teams, for their able stewardship throughout the negotiation process.

The political declarations represent an opportunity for the international community to recommit to the prevention of, better preparation for and more rapid and equitable responses to future pandemics, including the mainstreaming of pandemic preparedness. We have unfortunately fallen short of that goal. We were not able to reach consensus on important language that speaks to ensuring equitable access to health-care products, as well as references to stronger commitments, among other things, regarding technology transfer, removing intellectual property barriers and investing in manufacturing to enable local production in the global South.

We were also unable to reach agreement on the inclusion of language urging against the promulgation of unilateral coercive measures, as outlined in the 2030 Agenda for Sustainable Development. My delegation has long held the position that unilateral coercive

measures are a punitive measure that constitute one of the greatest obstacles to the implementation of the 2030 Agenda for Sustainable Development.

We would therefore like to emphasize that what we adopted today is a minimum, and we do not take it as the limit of our ambition. We emphasize that particularly as it pertains to the ongoing processes of the intergovernmental negotiating body to draft and negotiate a convention, agreement or other international instrument under the Constitution of the World Health Organization to strengthen pandemic prevention, preparedness and response. That is also applicable to the process to consider proposed amendments to the International Health Regulations in Geneva.

Ms. Carty (United States of America): The United States is pleased to join the consensus on the political declarations of the General Assembly high-level meetings on tuberculosis (resolution 78/5), universal health coverage (resolution 78/4) and pandemic prevention, preparedness and response (resolution 78/3). We appreciate the constructive approach of all three sets of co-facilitators — Poland and Uzbekistan, Guyana and Thailand, and Israel and Morocco — and thank General Assembly Presidents Csaba Kőrösi and Dennis Francis for their leadership in helping us to reach consensus.

The United States is a proud leader and the largest donor with regard to global health, with more than \$10 billion of assistance provided annually. Since his first day in office, President Biden has made a strong commitment to ensuring that our country is better prepared for future pandemics. The United States has repeatedly reaffirmed its long-standing commitment to strengthening global health security and its recognition of the need to do more to ensure that our partners around the world are better prepared to address future health threats.

Promoting and protecting the human rights of all persons in all their diversity is fundamental to achieving universal health coverage, creating a strong pandemic prevention, preparedness and response architecture and ending tuberculosis. That includes implementing laws, policies and programmes that eliminate stigma and discrimination and provide for the full inclusion of all persons.

To that end, we must include the voices of all women, girls, adolescents, lesbian, gay, bisexual, transgender, queer, intersex and asexual plus (LGBTQIA+) persons,

persons with disabilities, indigenous peoples and other marginalized and underrepresented populations in our decision-making. We must meaningfully include all persons in every aspect of planning, implementation, monitoring and accountability. We must reject policies that hinder their access to care because of bias, discrimination or stigma. Such actions are essential to meeting the Sustainable Development Goals (SDGs) and to building a more resilient world in the face of threats such as climate change, future pandemics, conflicts and crises.

On universal health coverage, the United States applauds the continued efforts to advance that priority — an essential building block towards achieving global health equity and strengthening global health security, including preventing, preparing for and responding to future pandemics. We join the global community in urging action to get back on track in all critical universal health coverage areas and in restoring essential health services to levels that are better than before the pandemic, ideally by 2025. This year's universal health coverage declaration has made some important progress on those elements since 2019, including strong language on health-care workers and access to universal health coverage for members of marginalized populations. The declaration also highlights the shortfalls of achieving universal health coverage on a global level by 2030, including where SDG 3 implementation has failed to progress since 2019.

The world has been profoundly impacted by the coronavirus disease (COVID-19) pandemic, with nearly 1 billion people infected and more than 7 million lives lost, along with devastating effects on economies, livelihoods and health systems. The pandemic demonstrated that, as a global community, we were underprepared. We need to address the gaps exposed by COVID-19 and prepare for the future. The United States is incorporating the lessons learned from COVID-19 on an ongoing basis, and we will continue to do our part to prepare for, and respond to, the next health emergency. This year's political declaration, the first of its kind in the General Assembly, laid down an important marker and reaffirmed at the Head of State level the importance of further enhancing political momentum and commitment towards pandemic prevention, preparedness and response, while remaining aligned with the Geneva-based processes on the intergovernmental negotiating body and the International Health Regulations.

The high-level meeting on fighting tuberculosis comes at a critical time. While the world continues to take steps towards universal health coverage and works to improve pandemic prevention, preparedness and response, the age-old scourge of tuberculosis (TB) takes more than 1.6 million lives each year. The United States remains the largest bilateral donor to the global effort to end TB, which has saved over 75 million lives to date. For more than two decades, we have worked with partners to end TB through surveillance, programmes and research efforts. We will continue those collaborations, including through our contributions to the Global Fund to Fight AIDS, Tuberculosis and Malaria. This year's political declaration on TB makes important progress on updating the language on targets and references to the Global Fund and Stop TB Partnership and incorporates important additional language on human rights, including a reference to the Universal Declaration of Human Rights.

We regret that we missed the opportunity to strengthen prior commitments on some critical cross-cutting issues. For example, advancing and respecting sexual and reproductive health and rights for all remain foundational to promoting gender equality and global health, and also protect the human rights of all persons in all their diversity. The United States is disappointed that we could not reach agreement to go beyond previously agreed language.

In addition, we regret that the phrase "multiple and intersecting forms of discrimination", as well as the fully inclusive listing of those most marginalized, including LGBTQIA+ persons, does not appear in the final versions of all three documents. However, we applaud the inclusion of strong language on the importance of, and challenges faced by, the global shortage of health workers in all three documents.

As we adopt the three political declarations in the General Assembly plenary today, the United States wishes to reiterate its outstanding concerns with the paragraphs on trade and technology transfer and the terminology around equal pay for equal work.

Mr. Van Schalkwyk (South Africa): South Africa would like to make the following explanation of position after the adoption of the three political declarations on pandemic prevention, preparedness and response (resolution 78/3), universal health coverage (resolution 78/4) and tuberculosis (resolution 78/5). Allow me first to thank the co-facilitators of the three

political declarations for their hard work during the difficult negotiation processes.

South Africa aligns itself with the statement made by the representative of Cuba on behalf of the Group of 77 and China.

Our leaders participated in the high-level meetings as a demonstration of the commitment that health, and access to it, is a human right and should not result in financial hardship or be determined by income levels. They also demonstrated their commitment to swiftly tackling pandemics and to ending tuberculosis by 2030. They reaffirmed the importance of health to the achievement of the Sustainable Development Goals. Indeed, health is an investment in human capital and social and economic development towards the full realization of human potential and significantly contributes to the promotion and protection of human rights and dignity, as well as the empowerment of all people.

The three high-level meetings on health were very timely and took place in the aftermath of the coronavirus disease (COVID-19) pandemic. Through the lessons learned during COVID-19 and its effects, it is very clear that, when addressing health emergencies and challenges, due recognition must be given to equity, unhindered access to quality and affordable medicines, including diagnostics, therapeutics and vaccines through the transfer of technology, and capacity-building in research, development and innovation, as well as local manufacturing and proper financing.

The pandemic has demonstrated that global health is inherently unequal and that national interest is prioritized over global commitments. Vaccine hoarding highlighted the fact that developing countries need unhindered access to health products, financing and technology transfer and capacities so that they can sustainably respond to any possible outbreaks, pandemics and other health crises. In addition, the impact of unilateral coercive measures demonstrated the lack of humanity during a health emergency and humanitarian crisis through indirect limitations on access to life-saving resources and medical countermeasures.

South Africa participated in the negotiations on the three health outcomes through the Group of 77 and China and the Group of African States. We are concerned that the multilateral processes were undermined when pertinent issues were raised by the Groups and their views were largely ignored. It is regrettable that the three

political declarations failed to acknowledge the voices of the countries of the South on important elements. If it was not deemed necessary to acknowledge unilateral coercive measures in the three political declarations, at least the language on unhindered access to quality, affordable and therapeutic tools could have been recognized as a matter of compromise.

While we accept the adoption of the three political declarations, South Africa's position remains steadfast that those outcomes remain incomplete and have failed to recognize the needs and aspirations of developing countries.

Mr. Elshandawily (Egypt) (*spoke in Arabic*): Egypt's delegation welcomes the General Assembly's adoption of the three political declarations of the high-level meetings held in September, on pandemic prevention, preparedness and response (resolution 78/3), universal health coverage (resolution 78/5) and the fight against tuberculosis (resolution 78/5), reflecting international solidarity towards achieving Sustainable Development Goal 3. Egypt's delegation expresses its appreciation for the efforts of the co-facilitators of the negotiations on the draft texts.

Egypt remains committed to advancing international efforts to promote the right to the highest standard of universal health coverage and overcome the gaps that have been revealed by the coronavirus disease (COVID-19) pandemic in order to address the international inequalities exposed by the pandemic. In that context, Egypt continues its national efforts given our Vision 2030, which seeks to provide the highest standard of living for citizens, including health care.

Despite Egypt's participation, developing countries were looking forward to references in the three political declarations to a commitment to overcoming the obstacles facing countries, especially developing countries, and ensuring equal and unhindered access to the relevant necessary medical tools, equipment and technologies, as stated in resolution 77/275. However, we believe that the adoption of the political declarations represents an international commitment to supporting the efforts of developing countries to strengthen their national health systems.

With regard to the texts of the declarations adopted today, Egypt's delegation would like to place on record that, with regard to paragraph 52 of the political declaration on pandemic prevention, preparedness and response and paragraph 62 of the political declaration

on universal health coverage regarding sexual and reproductive health, reproductive rights and sexual and reproductive health-care services, our interpretation of both paragraphs is based on the Programme of Action of the International Conference on Population and Development, held in Cairo. That interpretation is consistent with Egyptian national laws and legislation and with the values of Egyptian society.

Mr. Gaal (Hungary): Hungary welcomes the adoption by consensus of the three political declarations of the high-level meetings on health-related matters (resolutions 78/3, 78/4 and 78/5), held back-to-back in September during the high-level week of the General Assembly. We would like to convey our appreciation to the facilitators of the various processes for their tireless efforts, ensuring that we, as the international community, arrived at concise and action-oriented political declarations.

Hungary is steadfastly committed to the realization of the Sustainable Development Goals, including ensuring healthy lives and promoting the well-being for all at all ages. We are of the view that pandemic prevention and response should be based on professional experience. Saving lives cannot be a choice of politics or ideology. Governments around the world have an overarching responsibility to prepare their health-care systems for public health emergencies, including pandemics, and to ensure the achievement of universal health coverage.

Concerning the respective political declarations on pandemic prevention, preparedness and response and on universal health coverage, we welcome the fact that the texts make an explicit reference to the primary role and responsibility of Governments at all levels to determine their own path towards achieving the goals set out in those documents. Therefore, when it comes to the implementation of those political declarations, Hungary reserves the right to apply the provisions of the documents in line with its prevailing national laws and legislation, in particular with regard to paragraphs 7 and 34 of the political declaration on pandemic prevention, preparedness and response and paragraphs 23, 48 and 65 of the political declaration on universal health coverage.

We believe that national health-care policies and priorities must remain a matter for Member States' Governments to set and regulate. In that vein, access to national health-care systems for individuals belonging

to different groups of people, for example migrants, should be based on nationally determined policies and should be in accordance with the applicable provisions of international law. It is our firm position that no provisions of the declarations should be read as originating any newly recognized rights in that regard.

Ms. Del Águila Castillo (Guatemala) (*spoke in Spanish*): Guatemala thanks the delegations of Poland, Uzbekistan, Israel, Morocco, Guyana and Thailand for co-facilitating the three health processes, which involved several weeks of hard work.

I wish to refer to resolutions 78/3 and 78/4, adopted at this plenary meeting and containing the political declarations adopted during the high-level meetings held on 20 and 21 September, respectively.

My country reaffirms its commitment to continuing to work to achieve universal health coverage. However, Guatemala dissociates itself from paragraph 52 of the political declaration on pandemic prevention, preparedness and response and from paragraph 62 of the political declaration on universal health coverage, expanding our ambition for health and well-being in a post-coronavirus disease world and taking into account the fact that that the language contained in both paragraphs of the declarations that I just mentioned contravenes national legislation and public policy on the protection of life and the institutional framework of the family in Guatemala.

Guatemala promotes, defends and protects the human rights of all persons without any discrimination based on a principle of conventionality. The country therefore has reservations about the application of terms, conditions and provisions that are explicitly or implicitly contrary to the Political Constitution of the Republic of Guatemala and its internal legal system, including, but not limited to, issues related to sexual and reproductive rights.

We also reserve the right to interpret the term “reproductive rights”, which for the State of Guatemala does not include abortion. There is no international consensus on the interpretation of reproductive rights. Guatemala has national legislation in place that provides for only sexual and reproductive health policies, not reproductive rights, which could be interpreted as a right to abortion or abortion practices, which contravenes the country’s national legislation.

Furthermore, my delegation wishes to emphasize that the States members of the World Health Organization (WHO) agreed to launch a global process to draft and negotiate a convention, agreement or other international instrument within the framework of the WHO Constitution to strengthen prevention, participation and response to pandemics, which is still under negotiation. In that regard, this political declaration should not undermine the process under way in Geneva, but should be an element that contributes to it.

Mr. Woodroffe (United Kingdom): Like other speakers, I would like to begin by thanking both the former and current Presidents of the General Assembly and the co-facilitators for their hard work, which enabled the adoption of political declarations on pandemic prevention, preparedness and response (resolution 78/3), universal health coverage (resolution 78/4) and the fight against tuberculosis (resolution 78/5) this morning. We are pleased that, after many months of negotiations, we were able to reach consensus on our collective ambition here at the General Assembly following the endorsement of the declarations at the highest levels during their respective high-level meetings.

The United Kingdom is committed to promoting the health and well-being of all and to advancing progress on Sustainable Development Goal 3. We stand ready to continue to play our part in implementing the commitments set out in the declarations.

During the high-level meetings, the United Kingdom announced a package of almost £500 million in support for global health. That includes £295 million to respond to disease outbreaks and improve maternal and child health in low- and middle-income countries, £103 million to halt the spread of infectious diseases and support sexual and reproductive health, £95 million to tackle deadly diseases in Africa and £5 million to combat tuberculosis (TB). That funding will contribute to strengthening health systems, boosting collaboration on research and development, improving coordination across human, animal and environmental health sectors and improving access to safe and effective vaccines, medicines and tests.

We welcome the increased focus on scaling up investment in primary health care and TB. We must reform the international financial system to release more finance for health systems and pandemic preparedness in developing countries, including through greater investments in the Pandemic Fund. However, domestic

public resources must be the primary source for such stronger health systems, alongside enhanced international support.

We also encourage progress on negotiations on the pandemic accord in Geneva to agree measures to improve pandemic prevention, preparedness and response, while respecting national sovereignty.

The most recent universal health coverage global monitoring report paints an alarming picture, and we welcome the commitment to faster global progress to enable wider service coverage, financial protection and stronger health workforces.

While TB still kills more people than any other infectious disease, we welcome the commitment to redoubling efforts to get back on track to end TB by 2030, which will also help us in the fight against anti-microbial resistance (AMR).

We are, however, frustrated that the declarations did not move us forward on gender equality and sexual and reproductive health and rights, both of which are integral to achieving universal health coverage and our wider health goals for all. Despite that, we hope that the declarations will act as a platform from which we can all move forward with renewed ambition, cooperation and partnership to achieve health and well-being for all.

Finally, next year's high-level meeting on anti-microbial resistance will be a landmark opportunity to tackle AMR on a truly global scale. We look forward to playing our part in that as we continue to work together to advance global health.

Mr. Nakagawa (Japan): We would like to welcome today's consensus adoption of the political declarations of the high-level meetings on pandemic prevention, preparedness and response (resolution 78/3), universal health coverage (resolution 78/4) and the fight against tuberculosis (resolution 78/5). We would also like to extend our deep appreciation for the dedicated efforts of the co-facilitators — Morocco and Israel, Thailand and Guyana, and Poland and Uzbekistan, respectively — as well as the able leadership of both the former and current Presidents of the General Assembly.

We find ourselves at a pivotal juncture in the wake of the coronavirus disease pandemic. The lessons that we have learned must not be forgotten. It is incumbent upon the international community to rekindle the spirit of its collective goal to leave no one's health behind and prepare for the next pandemic.

Our journey towards universal health coverage has been fraught with difficulties. The challenges have become increasingly complex, with divisions surfacing within the international community. It is during those challenging moments that we must return to the core of our mission, encapsulated in the simple, yet profound principle of leaving no one behind. We must illuminate the path forward with the beacon of human dignity, ensuring that our commitment to those principles remains steadfast. Yet, in the pursuit of consensus, we acknowledge that challenges emerged during the negotiations, particularly concerning specific policy issues, such as technology transfer. Japan underscores the importance of addressing such issues in the forums and institutions specifically mandated for such discussions, in line with the respective governance for frameworks.

Today's adoption of the health-related political declarations marks a midpoint in our journey towards the 2030 Agenda for Sustainable Development. Japan remains resolute in its commitment to the concept of human security, and we continue to redouble our efforts in pursuit of Sustainable Development Goals, leaving no one behind.

In conclusion, let us remember that our work is far from over. The challenges ahead may be formidable, but our dedication to the principles of human dignity, solidarity and cooperation will guide us towards a future where all people can access the quality health care that they deserve.

Mr. Nicolino (Argentina) (*spoke in Spanish*): My delegation makes this statement on resolution 78/3 in its national capacity.

Argentina supports the adoption of that declaration on the understanding that it represents a step forward in the preparedness for, prevention of and response to future pandemics. We stress the importance of equity as a guiding principle. Cooperation and solidarity are the two central pillars that will make it possible to tackle the challenges ahead and prepare for another possible health emergency.

In that regard, Argentina has supported every regional and global initiative aimed at facilitating universal, equitable and free, or sufficiently affordable, access to key supplies, such as vaccines, medicines and other technologies, especially in low- and middle-income countries. For Argentina, medicines, vaccines, treatments and other health technologies

developed in response to a health emergency are considered global public goods and an essential component of the right to health. In that regard, the principles of transparency and reasonableness should be followed in agreements in order to contribute to their universal access and equitable distribution.

Argentina also stresses the need to strengthen the financing and capacities of developing countries for future pandemic preparedness, prevention and response. In that regard, Argentina believes that the members of the World Trade Organization should act expeditiously to extend the decision of the twelfth Ministerial Conference on the Agreement on Trade-Related Aspects of Intellectual Property Rights to cover the production and supply of coronavirus disease therapeutics and diagnostics for, in accordance with paragraph 8 of that decision.

Finally, Argentina believes that technology transfer cannot be considered solely on a voluntary basis and on mutually agreed terms, as proposed in the current wording. That would imply that it would be on purely commercial terms, when it should be on preferential terms to developing countries in order to allow the domestic development of technologies by those countries over the long term.

Mr. Prabowo (Indonesia): At the outset, Indonesia would like to congratulate the President on the consensus adoption of the political declarations on pandemic prevention, preparedness and response (resolution 78/3), universal health coverage (resolution 78/4) and the fight against tuberculosis (resolution 78/5). Furthermore, we would like to also appreciate the work of the co-facilitators in navigating and leading the deliberations on the three political declarations.

As a country of more than 275 million people spread across more than 17,000 islands, the three political declarations play an eminent role in boosting the implementation of sustainable development in the health sector in Indonesia. While each of the specific topics holds significant importance, we also believe that the political declarations are interlinked one to another.

With that understanding, Indonesia played an active role in contributing its views to the formulation of the political declarations on pandemic prevention, preparedness and response, universal health coverage and the fight against tuberculosis. We are ready to support the implementation of those three political

declarations at all levels, including within the United Nations environment.

In that regard, Indonesia is also pleased to share that, together with Nigeria, the Philippines and Poland, we have initiated the alliance of countries on the fight against tuberculosis. We would like to invite countries to join that collective fight against tuberculosis.

Mrs. Birrer (Switzerland) (*spoke in French*): Switzerland attaches great importance to the political declarations. They represent an important step towards our shared vision of achieving the Sustainable Development Goals. We thank the co-facilitators for their tireless work.

In our view, the declarations are not perfect. In particular, Switzerland considers that the political declaration on pandemic prevention, preparedness and response (resolution 78/3) should not prejudge the outcome of the discussions still under way in Geneva. The outstanding differences in the wording of paragraph 43 on access and benefit sharing will have to be negotiated in an appropriate and legitimate manner in the processes under way at the World Health Organization for Switzerland to support its content. Nevertheless, Switzerland supports the current consensus, as we attach great importance to the overall objective of succeeding in formally adopting the political declarations. We regret that that objective was not achieved before the high-level week.

Attempts to modify texts after they have been approved by Heads of State and Government seem to us to be very worrisome. Reconsidering fundamental aspects of a declaration after the text has already been submitted to our leaders is extremely problematic.

In conclusion, we wish to express our concern over this process. We call on Member States not to allow this situation to set a new precedent, and we hope that such an approach will be avoided in our future negotiations.

Mr. Sahraoui (Algeria) (*spoke in Arabic*): At the outset, Algeria expresses its great satisfaction over the consensus among members of the General Assembly on the adoption of the three relevant resolutions 78/3, 78/4 and 78/5 without a vote, although we did not reach consensus on them during the negotiations. That highlights the deep gap among the various countries, especially the countries of the North and the South.

The adoption of the draft resolutions takes place in the wake of the coronavirus disease (COVID-19)

pandemic, which served to reiterate the importance of the need for the international community to give the utmost priority to health, which is pivotal to achieving sustainable development and building sound societies and a better world. Today, after concluding the negotiations, with all their complexities, it is time to unite our efforts in order to live up to the commitments that we agreed and turn them into a reality in order to achieve universal health coverage in a way that would strengthen our national health systems and ensure the right of all to health, while rendering the world more prepared to deal with such pandemics.

The negotiations on the health resolutions during the past period represented a challenge to all delegations, especially the delegations of developing countries, owing to the pressure to which they were subjected and the need for a comprehensive approach to all the negotiation tracks, which required greater organization and coordination among all stakeholders in the negotiation process.

That was emphasized by all delegations, even before starting the negotiations. The sixteenth preambular paragraph of resolution 77/275 highlights the importance of ensuring coordination in the organization of the high-level meeting on pandemic prevention, preparedness and response with the high-level meetings on universal health coverage and on tuberculosis. That was reflected in paragraph 6 of resolution 78/4, adopted today, which acknowledges the importance of coordination across health-related processes taking place during the seventy-eighth session of the General Assembly.

However, unfortunately, my country notes that resolution 78/4, on universal health coverage, and resolution 78/3, on pandemic prevention, preparedness and response, are not consistent in terms of transfer of technology. Therefore, Algeria dissociates itself from paragraph 39 of resolution 78/3 and stresses that the consensual language is that of paragraph 74 of resolution 78/4, which should be adopted in all other resolutions, especially those pertaining to health. The same applies to paragraph 52 of resolution 78/3, which is not in line with paragraph 62 of resolution 78/4, which reflects the consensual language used. Algeria therefore also dissociates itself from paragraph 52 of resolution 78/3 and rejects redrafting the language of the 2030 Agenda on Sustainable Development.

The family is the main unit of the society, and most national health systems are based on programmes that

take the positive role of the family into consideration. In that regard, Algeria regrets the deletion of the language on the role of the family in resolution 78/5, on tuberculosis, although it was consensual language. We hope that that will be avoided in other resolutions.

Algeria also deeply regrets the non-consensus on adopting language from decision SSA2(5) of the World Health Assembly, on the availability of some medical procedures without any obstacles. We believe that that would have ensured fairness in dealing with pandemics and would guarantee the rights of developing countries, free from unilateral protectionist policies, which increased during COVID-19.

In that context, Algeria is surprised that none of the three resolutions include language on the negative impacts of unilateral coercive measures, although several negotiating countries and groups requested that, in particular the Group of 77 and China and the Group of African States. We emphasize that the agreed language of the 2030 Agenda for Sustainable Development should be the basis for any future negotiations to achieve a consensus that takes into account the positions of all countries and their interests.

In conclusion, we hope that the resolutions adopted today will represent a new starting point towards further cooperation so that everyone can enjoy high-quality health services free of any severe financial impacts, particularly in developing countries. That requires greater cooperation by increasing funding and accelerating the transfer of technology towards countries of the South in order to promote their capacities and strengthen their national health systems.

Ms. Matos Menéndez (Dominican Republic) (*spoke in Spanish*): I should first like to refer to the negotiation process on the political declaration on pandemic prevention, preparedness and response (resolution 78/3). In that regard, the Dominican Republic would like to express its sincere gratitude to the co-facilitators of the political declaration, the Ambassadors of Morocco and Israel, for their unwavering commitment and leadership, which led to a successful outcome to that clearly challenging process.

Negotiations lasted over approximately four months, with prolonged days of dialogue, requiring many hours of the day in talks, both with our teams in Geneva and in our capital. However, given the newness of the topic, we found ourselves up against many challenges, including ensuring greater coordination

with other related processes. However, we managed to achieve a document that largely reflects the interests of all members, gradually overcoming the obstacles with perseverance.

It is essential to highlight the challenges that we faced during that process in order to promote better coordination in future negotiations, particularly regarding agreed language, using parallel negotiations, in which we also are involved. Despite the fact that we would have hoped for a better balance being struck regarding some of the elements in the declaration, particularly concerning matters of priority, the Dominican Republic was in favour and fully supported it. The text provides recommendations to improve a holistic, cross-cutting approach to the necessary measures to strengthen health systems and the institutions involved therein, making pandemic preparation a fundamental requisite.

I would like to take this opportunity to also express our thanks and congratulations to the Ambassadors of

Guyana, Thailand, Poland and Uzbekistan, who guided the drafting of the political declaration on universal health coverage (resolution 78/4) and the fight against tuberculosis (resolution 78/5). The Dominican Republic fully supports those declarations.

The Acting President: We have heard the last speaker in explanation of position after adoption.

On behalf of the President of the General Assembly, I would like to convey my sincere appreciation to the Permanent Representatives of Morocco, Israel, Guyana, Thailand, Poland and Uzbekistan and the respective teams of experts, who ably and patiently conducted the discussions and complex negotiations in the informal consultations on the political declarations. I am sure that members of the Assembly join him in extending sincere appreciation to them.

The General Assembly has thus concluded this stage of its consideration of agenda item 125.

The meeting rose at 12.15 p.m.